

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Ixiaro (Japanese Encephalitis Vaccine), Rabies Vaccine (Inactivated) and Meningitis ACWY Vaccine (Nimenrix/Menveo) for the treatment of Japanese Encephalitis, Rabies and Meningococcal Disease (Groups A, C, W-135, Y) Prevention in Travellers

Summary of NICE / NICE CKS Guidelines for Japanese Encephalitis, Rabies and Meningococcal Disease (Groups A, C, W-135, Y) Prevention in Travellers

Overview

Japanese encephalitis is a mosquito-borne viral infection found in Asia and the Pacific region, with highest transmission in rural and agricultural areas during monsoon seasons. Rabies is a fatal viral encephalitis transmitted through bites or scratches from infected animals, including dogs, bats, and monkeys in endemic areas. Meningococcal disease is a serious bacterial infection causing meningitis and septicaemia, particularly important for travellers to the sub-Saharan Africa meningitis belt and those attending Hajj or Umrah.

Assessment

- **Travel risk factors for Japanese Encephalitis:** Travel to endemic areas, especially rural regions, agricultural areas, and those requiring outdoor/evening activities. Transmission is seasonal and more common during monsoon periods.
- **Travel risk factors for Rabies:** Travel to areas with endemic rabies and high risk of animal contact (outdoor activities, occupational exposure, adventure activities). Pre-exposure prophylaxis is recommended for high-risk travellers.
- **Travel risk factors for Meningococcal Disease:** Travel to the sub-Saharan Africa meningitis belt, especially during the dry season (November to May). Also recommended for pilgrims attending Hajj or Umrah.

All three vaccines are inactivated vaccines suitable for immunocompromised patients. Live vaccines are contraindicated in immunocompromised individuals but these products are not live vaccines.

Management

- **Japanese Encephalitis Vaccination:** Two doses of Ixiaro, 28 days apart. Booster at 12-24 months if continued risk. Can be given with other vaccines at different anatomical sites.
- **Rabies Pre-exposure Prophylaxis:** Three-dose schedule on days 0, 7, and 21-28. Intradermal regimen is available in some settings. Post-exposure prophylaxis and immunoglobulin required if exposure occurs.
- **Meningitis ACWY Vaccination:** Single dose for travellers. Protection typically begins after 10-14 days. Revaccination may be required after 3-5 years if risk continues.

All three vaccines should be administered intramuscularly. Ensure informed consent is obtained and documented. Patients should be advised to seek urgent medical attention if fever, severe headache, or rash develops within 30 days of vaccination.

Red Flags and Contraindications

- **Absolute contraindications:** Previous anaphylactic reaction to any component of the vaccine.
- **Caution in:** Pregnancy - inactivated vaccines are generally safe but assessment of risk vs benefit is needed. Acute febrile illness - defer vaccination. Guillain-Barré syndrome in past - assess risk carefully.
- **Immunocompromised patients:** Inactivated vaccines like these may have reduced efficacy but can still be given. Live vaccines must be avoided. Seek specialist advice if severely immunocompromised.

Patient Group Direction

for the supply/administration of

Ixiaro (Japanese Encephalitis Vaccine, Inactivated, Adsorbed)

for the treatment of

Japanese Encephalitis, Rabies and Meningococcal Disease (Groups A, C, W-135, Y) Prevention in Travellers

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Prevention of Japanese encephalitis in travellers aged 18 years and over to endemic areas
Inclusion criteria	Adults aged 18 years and over

	Travelling to endemic areas (Asia-Pacific region with high transmission risk) Valid informed consent obtained No absolute contraindications
Exclusion criteria	Age under 18 years (outside scope of this PGD; seek specialist advice) Previous anaphylactic reaction to any component of Ixiaro Hypersensitivity to formaldehyde or protamine sulphate Acute febrile illness with temperature exceeding 38.5°C Pregnancy - seek specialist advice as benefits may outweigh risks Administration of live vaccines within 4 weeks (if applicable)
Cautions	Recent receipt of another inactivated vaccine (can be given at same visit but different site) History of Guillain-Barré syndrome Bleeding disorders or anticoagulation therapy - seek specialist advice Severe immunosuppression - may require specialist assessment but vaccine is still indicated Breastfeeding - unlikely to pose significant risk but consider timing Known allergy to eggs (minimal risk as produced in Vero cells)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Ixiaro injection, 0.5 ml prefilled syringe, containing 6 micrograms inactivated Japanese encephalitis virus antigen (Vero cell derived)
Legal category	POM (Prescription Only Medicine)
Storage	Store at 2-8°C. Do not freeze. Keep in original packaging to protect from light.
Route/method of administration	Intramuscular injection into deltoid muscle of upper arm. Aspirate recommended before injection to avoid intravascular injection.
Dose and frequency	Primary course: 0.5 ml on day 0, then 0.5 ml on day 28 Booster: 0.5 ml at 12-24 months if ongoing risk of exposure
Quantity to be administered and/or supplied	0.5 ml per dose (one prefilled syringe)
Maximum or minimum treatment period	Primary course: 28 days minimum between doses. Booster administered 12-24 months after completion of primary course

Adverse effects	Very common ($\geq 1/10$): Injection site pain, injection site tenderness, injection site erythema Common ($\geq 1/100$): Headache, myalgia, fatigue, nausea, fever Uncommon ($\geq 1/1000$): Dizziness, lymphadenopathy Rare: Anaphylaxis, angioedema, Guillain-Barré syndrome (temporal association unclear) Advise patient to seek medical attention if severe reactions occur or if symptoms develop within 4 weeks
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner - name and brand of medication - date of supply dose, form and route quantity supplied - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: - Adults aged 18 years and over - keep records for 8 years - Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflets (PILs) provided with each vaccine. Ensure patient understands the need for rabies post-exposure prophylaxis and immunoglobulin if animal exposure occurs despite prophylaxis.
Follow-up advice to be given to patient or carer	Seek medical advice if fever, severe headache, stiff neck, or rash develops within 30 days of vaccination. If bitten or scratched by an animal while travelling, immediately wash wound with soap and clean running water for 15 minutes, apply local antiseptic, and seek urgent medical attention for post-exposure prophylaxis. Rabies vaccine alone does NOT provide full protection without

	immunoglobulin if exposure occurs; post-exposure rabies immunoglobulin must be given. Recall appointments for Japanese encephalitis booster at 12-24 months if travel risk continues. Meningitis ACWY protection typically begins within 10-14 days; ensure vaccination completed at least 10 days before travel if possible. Report any adverse effects via the Yellow Card scheme or to the GP. Maintain personal precautions against mosquitoes (Japanese encephalitis) and avoid stray animals (rabies) while travelling. Ensure travel insurance covers any required medical treatment related to these vaccine-preventable diseases.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Travel Health and Vaccinations
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

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Patient Group Direction

for the supply/administration of

Rabies Vaccine, Inactivated (Rabipur or equivalent)

for the treatment of

Japanese Encephalitis, Rabies and Meningococcal Disease (Groups A, C, W-135, Y) Prevention in Travellers

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Pre-exposure rabies prophylaxis for travellers to rabies-endemic areas with risk of exposure to infected animals
Inclusion criteria	Adults aged 18 years and over and children with appropriate parental consent Travelling to rabies-endemic areas with occupational or activity-related risk No previous rabies vaccination or with low antibody titres requiring booster Valid informed consent obtained No absolute contraindications

Exclusion criteria	Previous anaphylactic reaction to rabies vaccine or any component Hypersensitivity to neomycin or other antibiotics in the formulation Acute febrile illness with temperature exceeding 38.5°C Severe immunosuppression (relative contraindication - seek specialist advice) Administration of live vaccines within 4 weeks (if applicable)
Cautions	Chronic illness such as hepatic or renal disease - seek advice if significant Bleeding disorders or anticoagulation therapy - use smaller gauge needle or intradermal route Pregnancy - benefits likely outweigh risks; documented post-exposure rabies prophylaxis effectiveness in pregnancy Immunosuppressive therapy - may reduce immune response; serological testing may be indicated Previous allergic reaction to vaccine components (non-anaphylactic) - assess carefully Latex sensitivity - needle cover contains latex
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Rabipur or equivalent inactivated rabies vaccine, 1 ml vial containing 2.5 international units (IU) of inactivated rabies virus
Legal category	POM (Prescription Only Medicine)
Storage	Store at 2-8°C. Do not freeze. Keep in original packaging to protect from light. Discard if vaccine has been frozen.
Route/method of administration	Intramuscular injection into deltoid muscle of upper arm (preferred site). Anterolateral thigh muscle acceptable in small children. Do not inject intradermally for pre-exposure prophylaxis in this PGD.
Dose and frequency	Pre-exposure prophylaxis: 1 ml on day 0, 1 ml on day 7, 1 ml on day 21-28 Booster: 1 ml at 12-24 months if ongoing occupational risk or low antibody titre at testing
Quantity to be administered and/or supplied	1 ml per dose (one vial)
Maximum or minimum treatment period	Primary pre-exposure course: 3 weeks minimum between initial and final dose. Booster at 12-24 months if required.

Adverse effects	Very common ($\geq 1/10$): Injection site pain, injection site tenderness, injection site erythema Common ($\geq 1/100$): Headache, malaise, myalgia, nausea, dizziness, fever Uncommon ($\geq 1/1000$): Lymphadenopathy, fatigue Rare: Anaphylaxis, angioedema, Guillain-Barré syndrome, immune complex-like reactions Advise patient to seek medical attention if severe reactions occur. Post-exposure prophylaxis still required if exposure occurs despite prophylaxis
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflets (PILs) provided with each vaccine. Ensure patient understands the need for rabies post-exposure prophylaxis and immunoglobulin if animal exposure occurs despite prophylaxis.
Follow-up advice to be given to patient or carer	Seek medical advice if fever, severe headache, stiff neck, or rash develops within 30 days of vaccination. If bitten or scratched by an animal while travelling, immediately wash wound with soap and clean running water for 15 minutes, apply local antiseptic, and seek urgent medical attention for post-exposure

	prophylaxis. Rabies vaccine alone does NOT provide full protection without immunoglobulin if exposure occurs; post-exposure rabies immunoglobulin must be given. Recall appointments for Japanese encephalitis booster at 12-24 months if travel risk continues. Meningitis ACWY protection typically begins within 10-14 days; ensure vaccination completed at least 10 days before travel if possible. Report any adverse effects via the Yellow Card scheme or to the GP. Maintain personal precautions against mosquitoes (Japanese encephalitis) and avoid stray animals (rabies) while travelling. Ensure travel insurance covers any required medical treatment related to these vaccine-preventable diseases.
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Key references

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Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Meningitis ACWY Vaccine, Conjugate (Nimenrix or Menveo)

for the treatment of

Japanese Encephalitis, Rabies and Meningococcal Disease (Groups A, C, W-135, Y) Prevention in Travellers

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Prevention of meningococcal disease (serogroups A, C, W-135, Y) in travellers to high-risk areas
Inclusion criteria	Adults aged 18 years and over and children as per ACWY guidance Travelling to sub-Saharan Africa meningitis belt, especially during dry season Pilgrims attending Hajj or Umrah Documented meningitis vaccination status checked Valid informed consent obtained No absolute contraindications
Exclusion criteria	Previous anaphylactic reaction to meningococcal ACWY vaccine or

	any component Hypersensitivity to latex (if using products with latex-containing needles) Acute febrile illness with temperature exceeding 38.5°C Recent receipt of another meningococcal vaccine (minimally 2 weeks apart recommended)
Cautions	History of Guillain-Barré syndrome - seek specialist advice Severe immunosuppression - may require specialist assessment but vaccine still indicated; antibody response may be reduced Asplenia or complement deficiency - seek specialist advice regarding additional prophylaxis Pregnancy - benefits likely outweigh risks; consultation with GP recommended Breastfeeding - no known contraindication Recent receipt of inactivated vaccine (not live) - can be given at same visit but different anatomical site
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Nimenrix or Menveo: meningococcal groups A, C, W-135 and Y polysaccharide conjugate vaccine, 0.5 ml prefilled syringe or vial
Legal category	POM (Prescription Only Medicine)
Storage	Store at 2-8°C. Do not freeze. Keep in original packaging to protect from light. Discard if exposed to freezing.
Route/method of administration	Intramuscular injection into deltoid muscle of upper arm. Anterolateral thigh muscle acceptable in infants and young children. Aspirate not required.
Dose and frequency	Single dose of 0.5 ml intramuscularly
Quantity to be administered and/or supplied	0.5 ml per dose (one prefilled syringe or vial)
Maximum or minimum treatment period	Single-dose vaccination. Booster may be required after 3-5 years if risk of exposure continues or if travel plans dictate.
Adverse effects	Very common ($\geq 1/10$): Injection site pain, injection site erythema, injection site swelling Common ($\geq 1/100$): Headache, fatigue, nausea, myalgia, fever

	Uncommon ($\geq 1/1000$): Dizziness, lymphadenopathy, diarrhoea Rare: Anaphylaxis, angioedema, Guillain-Barré syndrome (temporal relationship unclear) Advise patient to seek medical attention if severe reactions occur. Fever and myalgia may be minimised by paracetamol taken 30 minutes after vaccination (not as pre-medication)
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflets (PILs) provided with each vaccine. Ensure patient understands the need for rabies post-exposure prophylaxis and immunoglobulin if animal exposure occurs despite prophylaxis.
Follow-up advice to be given to patient or carer	Seek medical advice if fever, severe headache, stiff neck, or rash develops within 30 days of vaccination. If bitten or scratched by an animal while travelling, immediately wash wound with soap and clean running water for 15 minutes, apply local antiseptic, and seek urgent medical attention for post-exposure prophylaxis. Rabies vaccine alone does NOT provide full protection without immunoglobulin if exposure occurs; post-exposure rabies immunoglobulin must be given.

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